

PM-JAY Uttar Pradesh -- Analytical Findings Report

Automated MetaInsight Analysis

Claims Processing & Treatment Patterns

Spending is concentrated in a few specialties

The clearest finding is that PM-JAY spending in Uttar Pradesh is heavily concentrated in two specialties. Cardiology (CARD) alone accounts for about **■401.9 crore** or **39.4%** of all claimed amount, and Orthopaedics (ORTH) adds another **■260.2 crore** or **25.5%**. Together, these two specialties make up roughly **65% of total claimed spending** across the 11 specialties reviewed.

This matters operationally because any change in claim behaviour, package pricing, or medical audit practice in these two areas will have the biggest budget impact. Cardiology is also the largest package-cost driver by standard base amount: the approved base amount for Cardiology is **■31.43 crore** or **42.7%** of total base amount, compared with Orthopaedics at **■17.77 crore** or **24.1%**. The next largest areas are Obstetrics & Gynaecology (OBG) at **■8.50 crore** claimed and General Surgery (GS) at **■7.22 crore**. ENT is the smallest spending specialty at **■1.17 crore**.

The same pattern holds across all 18 divisions, which suggests this is not a local issue in one area; it is a system-wide spending profile. The practical implication is that any review of high-cost claims should prioritise Cardiology and Orthopaedics first.

Claim processing is mostly smooth, but a sizeable share still needs intervention

For settled claims, the approval and payment process is largely working, but not perfectly clean. Across normal discharges, claim status is dominated by **APPROVED** cases at **■64.69 crore** or **72.2%** of claim amount, with **SETTLED** claims at **■10.96 crore** or **12.2%**. However, **QUERY_RAISED** claims still account for **■8.31 crore** or **9.3%**, while **REJECTED** claims are **■3.60 crore** or **4.0%**, and **PENDING** claims are **■2.03 crore** or **2.3%**.

At the pre-authorisation stage, the picture is similar: among settled claims, **APPROVED** pre-auths account for **■6.58 crore** or **51.4%**, **AUTO_APPROVED** for **■4.33 crore** or **33.8%**, while **QUERY_RAISED** still contributes **■1.54 crore** or **12.0%** and **REJECTED** **■0.35 crore** or **2.8%**. This tells us that a meaningful volume of cases still faces extra scrutiny before payment. Operationally, that may be appropriate for controls, but it also means frontline hospitals are spending time on clarifications, and district teams should watch whether query patterns are consistent and justified.

Turnaround time is fairly even across districts, but some districts are slower

For settled claims, average settlement time is broadly similar across districts, but there are still noticeable differences. The average is **9.28 days in Auraiya** at the low end and **15.09 days in Bahraich** at the high end. Other slower districts include **Basti (14.79 days)**, **Lalitpur (14.75 days)**, **Pilibhit (14.44 days)**, and **Jhansi (14.34 days)**.

The good news is that the overall pattern is even across districts, so this does not look like a widespread administrative failure. But the higher-TAT districts should be checked for avoidable delays in claim review, query closure, or approval routing. If a district is consistently above the state norm, it can affect hospital cash flow and may discourage timely settlement.

Most admissions end normally, and serious exits are relatively rare

Across the 22,500 treatment cases, **NORMAL** discharge accounts for **19,650 cases** or **87.3%**. The remaining cases are **LIVE** (ongoing) at **6.2%**, **LAMA** at **4.0%**, **DAMA** at **2.1%**, and **DEATH** at **0.4%**. This pattern is consistent across all divisions and also within settled claims, where normal discharge remains the dominant outcome at **2,549 cases** or **86.5%**.

From a programme perspective, this is a useful reassurance that most admissions are completing as expected. The small but visible share of LAMA/DAMA cases may still warrant local review, especially in facilities with higher clinical turnover or patient dissatisfaction.

Seasonal admissions and patient mix are stable, with a summer peak

Admission volume shows a consistent seasonal pattern across hospital types and time periods, with the highest activity in **August 2023 (818 cases)**, **August 2025 (777)**, **July 2023 (776)**, **July 2024 (767)**, and **July 2025 (748)**. The lowest month was **February 2023 with only 20 cases**, which is unusually low and should be checked for data completeness or reporting gaps rather than treated as a normal seasonal trough.

By disease mix, communicable diseases account for the largest number of cases at **6,524** or **29.0%**, followed by NCDs at **5,830** or **25.9%**, maternal and neonatal conditions at **4,149** or **18.4%**, surgical cases at **3,833** or **17.0%**, and injuries at **2,164** or **9.6%**. Injury is the smallest group, which is notable given its typically urgent nature.

Follow-up questions to take into district review

1. Why are **Bahraich, Basti, Lalitpur, Pilibhit, and Jhansi** taking longer than other districts to settle claims, and is the delay linked to query closure, staffing, or hospital documentation quality?
2. Are the high-value specialties — especially **Cardiology (CARD)** and **Orthopaedics (ORTH)** — generating proportionately more queries or rejections than other specialties?
3. What explains the very low admission volume in **February 2023**, and do any hospitals or districts have missing reporting for that month?

District-Level Monthly Performance Trends

Overall headline: enrolment activity rose through 2023, then weakened sharply in 2024–2025

Across Uttar Pradesh, monthly enrolment of new beneficiaries and new households was strongest in 2023 and then fell away in the later period. At the state-wide level, new beneficiaries peaked in 2023 at 88,591, which was 43.0% of the full period total, before dropping to 57,973 in 2024 and just 13,129 in 2025. New households show the same shape: 83,025 in 2023, or 42.5% of the total, compared with 55,394 in 2024 and 12,918 in 2025. In plain terms, the scheme's enrolment momentum was front-loaded and then slowed materially, which is important because lower enrolment will eventually constrain utilisation and claims growth.

The one district-specific exception worth noting is that the monthly claims-per-1,000-beneficiaries pattern is very even across all 75 districts overall, but the level still varies. **Amroha** is the highest on average at 3.79 claims per 1,000 beneficiaries, followed by Mathura (3.66), Kasganj (3.61), Siddharthnagar (3.60), and Sultanpur (3.54). On the lower side, **Sant Ravidas Nagar** averages 2.66 and **Hamirpur** 2.59. These are not huge gaps, but they are useful for identifying districts that may need a closer look at access, provider availability, or claim submission practices.

Enrolment and card issuance: the key operational concern is the post-2023 drop

The same decline is visible in Ayushman card-related activity and cumulative enrolment growth. Cumulative beneficiaries increased steadily in every district and at every reporting grain, which is expected for a running total. However, the pace of growth is clearly stronger at the quarterly and monthly level in the earlier part of the period than later on. This suggests that the programme expanded steadily, but recent enrolment effort has not kept up.

A notable exception in timing appears in the broader monthly change patterns: most activity measures shifted around March 2023, but new beneficiaries, new households, and cards issued show a later change point in **June 2025**. That means the slowdown in enrolment may not have been gradual everywhere; it may have changed after a specific operational event, campaign, or reporting shift in mid-2025. This deserves district and state-level review.

Utilisation and claims: admissions rose to a peak in 2023, with a clear seasonal pattern

Hospital admissions under PM-JAY also show a consistent seasonal pattern across districts, with the highest activity clustered around the same period each year. Monthly admissions were strongest in 2023, with the top month reaching 818 cases, and other high months including 2025-08 (777), 2023-07 (776), 2024-07 (767), and 2025-07 (748). The clearest operational reading is that utilisation is not flat through the year; claims and admissions rise in predictable seasonal windows, so staffing

and claim-processing capacity should be reinforced ahead of those months.

The quarterly claims-per-1,000-beneficiaries pattern changed around **2023Q1 in 74 districts**, with **Balrampur** being the exception, where the change was seen in **2023Q2** instead. This is important because it suggests a system-wide shift in utilisation or reporting from early 2023 onward, but with Balrampur behaving differently. That district may have had delayed rollout, delayed provider onboarding, or delayed beneficiary access compared with the rest of the state.

Claim processing and financial flow: most indicators move with the same seasonal cycle, but payment-related measures need separate attention

Most monthly measures tied to claim volume and financial flow — including claims submitted, claims approved, claims rejected, amounts claimed, amounts approved, and amounts paid — follow the same seasonal pattern seen in admissions. That is useful operationally because it means payment workload can be anticipated. However, approval rate does not follow the same simple seasonal shape, so it should be tracked separately rather than assumed to move with volume.

Likewise, emergency share, death rate, and average claim amount do not follow the same general pattern as volume measures. These indicators need their own monitoring because they may reflect case-mix changes, referral behaviour, hospital practice, or claim scrutiny rather than simple enrolment growth.

What to look into next

The main questions I would recommend you investigate are:

1. What happened around **June 2025** that coincided with the drop in new beneficiaries, new households, and cards issued?
 2. Why is **Balrampur** the only district where the quarterly utilisation shift came in **2023Q2** rather than **2023Q1**?
 3. Why do districts like **Amroha** and **Mathura** show higher claims per 1,000 beneficiaries than districts such as **Hamirpur** and **Sant Ravidas Nagar** — is this due to access, disease burden, hospital mix, or claim behaviour?
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Hospital Infrastructure & Specialty Capacity

Specialty mix is highly concentrated in Cardiology and Orthopaedics

The clearest finding is that PM-JAY spending in hospital-specialty services is dominated by two specialties everywhere we looked. Across all 18 divisions, **Cardiology (CARD)** and **Orthopaedics (ORTH)** are the top two specialties by claim value. Together they account for about **■658.2 crore** of the **■1,002.7 crore** claimed in this dataset, or roughly **65.6%** of total specialty claim value. Cardiology alone contributes **■400.4 crore (39.9%)**, and Orthopaedics contributes **■257.8 crore**

(25.7%). The next largest specialties are far behind: Obstetrics and Gynaecology (OBG) at **■84.9 crore (8.5%)**, General Surgery (GS) at **■72.2 crore (7.2%)**, and Urology (URO) at **■45.2 crore (4.5%)**.

Operationally, this means PM-JAY utilisation is heavily concentrated in a small number of high-value specialties. That may be expected to some extent, but it also means any weakness in Cardiology or Orthopaedics capacity, approval processes, or quality assurance will have a large effect on overall programme performance. At the other end, Burns and Psychiatry show **zero claim value** across the dataset, suggesting either very limited service availability, very low referral capture, or non-use under PM-JAY.

Hospital size and staffing are concentrated in larger facilities, but workload is not translating evenly into activity

Most bed capacity is concentrated in larger hospitals. Facilities in the **Large (101–300 beds)** category account for **594,988 beds**, or **62.9%** of all recorded bed strength. Very large hospitals (300+ beds) add another **163,126 beds (17.2%)**, while medium hospitals contribute **160,335 beds (16.9%)** and small hospitals only **27,537 beds (2.9%)**. In public hospitals, the pattern is different but still important: small facilities do not dominate bed capacity, yet they stand out on utilisation, as noted below.

Staffing levels are fairly even across specialty groups within each hospital type, but the average clinical staff count varies only slightly, from **5.13** to **5.52** staff across specialties. By hospital subtype, the average is highest in DERM (Dermatology) at **5.52** and lowest in CARD at **5.13**. This narrow range suggests that staffing counts alone may not explain major differences in service delivery. A closer review of staff mix, specialist availability, and duty patterns would be more useful than headcount alone.

Small public hospitals are carrying the highest case load per bed

The most operationally important utilisation pattern is in public hospitals. Across all 18 divisions, **small public hospitals (<=30 beds)** have the highest PM-JAY case load per bed, with an average of **0.22 cases per bed**, which is **73.9%** of the total among bed-size groups in this measure. Medium hospitals average **0.05 cases per bed (17.3%)**, large hospitals **0.02 (5.6%)**, and very large hospitals **0.01 (3.1%)**.

This tells us that the smallest public facilities are doing disproportionately more PM-JAY work relative to their bed base. That may indicate efficient use of local facilities, but it may also signal crowding, pressure on staff, or a tendency for cases to be absorbed at smaller centres rather than referred upward. Notably, this same pattern of small public facilities leading on cases per bed appears in every division, including **Varanasi, Moradabad, Basti, Gorakhpur, Prayagraj, Aligarh, Kanpur, Lucknow, Ayodhya, Chitrakoot, Meerut, Bareilly, Saharanpur, Azamgarh, Mirzapur, Devipatan, Jhansi, and Agra**.

License coverage is broadly similar, but service use is uneven and some specialties are barely active

Average active licensing levels are similar across hospital size groups in public facilities, ranging from **6.94** to **7.15** licenses on average. Expired license patterns are not singled out as a major exception here, but the dataset also shows that many hospital-specialty combinations recorded **zero claims**, which points to underuse of empanelled capacity even where licences and staff exist.

The clearest underused specialties by case intensity are Burns and Dermatology. For public hospitals, Burns and Dermatology both show **0.0 cases per bed** on average in the specialty-level comparison, despite being present in the hospital network. Across hospital subtypes, the same specialties sit at the bottom of the utilisation distribution. This is a practical signal that empanelment alone is not enough; some specialties are not turning into patient throughput.

Large hospitals dominate bed supply, but not all categories show the same claim pattern

When looking at bed strength by specialty, the largest share consistently sits in **Large (101–300 beds)** hospitals. They account for **594,988 bed-strength units (62.9%)** of the total specialty-linked bed capacity. Public hospitals show a different distribution: large hospitals contribute **■189,149** of bed strength, or **38.9%**, while very large hospitals contribute **■163,126 (33.5%)**. This suggests that the largest hospitals are important to the network, but they are not the only source of capacity.

What to look into next

1. Why are **Cardiology** and **Orthopaedics** absorbing nearly two-thirds of all claim value, and are these claims aligned with actual district-level disease burden and referral patterns?
 2. Why do **Burns** and **Psychiatry (PSYCH)** show no claims, and are these services genuinely unavailable, not empanelled effectively, or simply not being referred under PM-JAY?
 3. Are **small public hospitals** in districts like **Varanasi, Lucknow, Gorakhpur, and Agra** under pressure because they are carrying the highest case load per bed, and do they need tighter referral rules or additional capacity support?
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Beneficiary Enrolment & Scheme Uptake

Headline: uptake is strongest among better-documented, verified beneficiaries, while rejection and low card activation remain the main bottlenecks

Across the 205,847 enrolled beneficiaries in Uttar Pradesh, the data shows a clear operational pattern: people with stronger documentation and verified records are more likely to move from enrolment to actual scheme use. The biggest gap is not enrolment itself, but conversion into claims. That matters because enrolment only delivers value when the beneficiary can get the card, keep it active, and successfully use it at a hospital.

Documentation and card readiness are broadly uniform, but low-document groups need closer monitoring

Documentation quality is remarkably even across all four age groups and across divisions. The average number of identity documents per beneficiary is about 1.2 everywhere, with only very small differences between divisions: **Devipatan** and **Saharanpur** are highest at 1.21, while **Meerut** and **Aligarh** are slightly lower at 1.19. In practice, this suggests the documentation process is operating fairly consistently statewide, without one age group or division being clearly disadvantaged.

Card issuance is also broadly stable. For beneficiaries with only one document, the average time from enrolment to card issuance is 15.54 days for the 26–40 age group, 15.46 days for both 41–60 and 60+, and 15.45 days for 15–25. So the system is taking about two weeks to issue cards, and the waiting time does not vary much by age. That consistency is useful, but it also means there is no obvious operational shortcut being taken for some groups and not others. If card turnaround needs to improve, it will likely require a system-wide intervention rather than a district-specific fix.

Claims are concentrated in older age groups; younger beneficiaries are under-using the scheme

Claim activity is heavily skewed towards older beneficiaries. Of all claims filed, the 60+ group accounts for 8,649 claims, or 38.4% of total claims. The 41–60 group contributes 6,730 claims, or 29.9%. In contrast, the 15–25 group contributes only 2,028 claims, just 9.0% of all claims.

This is not just a volume issue; it likely reflects lower health service use, weaker awareness, or less successful navigation of the scheme among younger beneficiaries. The data does not show a problem with enrolment approval by age, but it does show that the youngest enrolled group is far less likely to generate claims than older groups. If the programme wants higher uptake, this is one place to look first.

SECC beneficiaries drive most claims, but card-status problems still block use

Claims are also concentrated by entitlement source. SECC beneficiaries account for 13,547 claims, or 60.2% of all claims. STATE_DB contributes 5,630 claims, or 25.0%, and RSBY contributes 3,323 claims, or 14.8%. This is important operationally because it shows the scheme is still depending mainly on the SECC base for utilisation.

Within active cards, verified “GOLD” records account for 11,089 claims, or 59.5% of all claims, compared with 5,648 claims from “SILVER” records and 1,906 from “PENDING” records. That means claim use is much stronger when records are fully verified. The implication is straightforward: incomplete record verification is likely reducing utilisation.

Card status is another major bottleneck. Among beneficiaries with only one document, 14,897 are active, but 837 are inactive and 491 are disabled. In claim terms, the pattern is similar: 15,739 claims come from active cards, while 870 are from inactive cards and 514 from disabled cards. Even though active cards dominate, the presence of claims from inactive or disabled cards suggests that status updates and enforcement may not be fully clean. That is worth checking because it can

create confusion at the hospital and raise grievance pressure.

Rejections are present statewide and should be treated as an operational warning sign

Rejections are not confined to a few places or a single beneficiary type. For active cards, the rejected category appears consistently across all districts, and statewide it accounts for 969 claims, or 5.2% of claims in that subgroup. Among beneficiaries with only one document, rejection still appears in the enrolment-status mix at 1,694 claims, or 9.9% of claims. This tells us that documentation constraints and rejection risk remain linked.

Operationally, this is a sign to inspect where beneficiaries are being stopped: at enrolment, during card creation, or later at the hospital interface. If rejections are concentrated among one-document beneficiaries, that is a strong cue to improve document support and on-ground correction before approval.

Follow-up questions to investigate

1. Which districts have the highest share of inactive or disabled cards among enrolled beneficiaries, and are those cards still being used at hospitals?
 2. Why is claim uptake so low among the 15–25 age group compared with older beneficiaries—awareness, illness burden, access, or enrolment quality?
 3. Where are rejected applications coming from most often: specific document types, specific entitlement sources, or specific districts?
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